

SPARTAN COACHING

DISCIPLINE | EMPATHY | STRATEGY

QUALIFYING DECISION TREES

Visual Logic Tools for Referral Source Conversations

Step-by-step decision frameworks for qualifying patient appropriateness, advancing stage conversations, and diagnosing stalled accounts.

Qualifying Decision Trees

Making the Right Call at Every Step

DECISION TREE 1: IS THIS ACCOUNT READY TO REFER?

Work through the following questions in order at any A or B Tier account. Each answer directs you to the appropriate next action. Do not skip questions.

Question 1: Have you had a meaningful discovery conversation?

- Yes, continue to Question 2
- No, action: Schedule a 15-minute discovery visit. Use the Stage 1 opener from the Cold Call Script. Do not try to advance past this step.

Question 2: Have you identified a specific pain point this account has?

- Yes, continue to Question 3
- No, action: You need one more discovery conversation focused on their challenges. Ask: "What is the hardest part of managing your most medically complex residents right now?"

Question 3: Have you delivered educational content specific to their pain point?

- Yes, continue to Question 4
- No, action: Schedule a lunch-and-learn or drop off a targeted resource. This is the bridge between Stage 2 and Stage 3. Educational value is the currency of trust in clinical settings.

Question 4: Has the contact responded positively to your educational content?

- Yes, continue to Question 5
- No response, action: Follow up with a direct phone call. Ask: "I sent over that resource last week — did you get a chance to glance at it? I wanted to hear your thoughts."
- Negative response, action: Ask what type of information they would find more useful. Pivot to their preferred learning format.

Question 5: Are there patients in this account who likely meet eligibility criteria right now?

- Yes, action: Ask directly — "Is there anyone you are managing right now who you have been wondering about in terms of appropriateness for comfort-focused care?"

- Unknown, action: Offer to conduct a brief case review with the clinical team. Phrase it as educational, not as a patient screening service.
- No, action: Maintain contact frequency and ask about patient mix and recent clinical trends during each visit.

DECISION TREE 2: WHY IS THIS ACCOUNT NOT REFERRING?

Use this tree when an account has the potential to refer but has gone quiet or is actively referring elsewhere. Work through each branch until you find the root cause.

Branch A: Competitor Is Entrenched

- Indicators: Account praises current provider, has long-standing relationship, never mentions dissatisfaction
- Strategy: Do not attack the competitor. Build your own relationship on the margin. Ask: "If there were ever a situation where your current provider wasn't the right fit for a particular patient or family, what would that situation look like?"
- Patience required: Competitor entrenchment typically takes 6 to 18 months to break without a service failure event. Focus on relationship depth while you wait for an opening.

Branch B: Service Quality Issue

- Indicators: Account was previously active but referrals dropped suddenly or gradually, contact is less warm in recent visits
- Strategy: Ask directly — "I want to make sure our team is giving you and your patients the level of support you expect. Has anything happened that I should know about?" Then listen without defending.
- Escalate: If there is a legitimate service concern, escalate it to clinical leadership the same day. Follow up with the account within 48 hours with a specific update on what is being done.

Branch C: Decision-Maker Has Changed

- Indicators: New DON, new discharge planner, new social worker at previously active account
- Strategy: Request an introduction meeting with the new decision-maker. Your relationship with the predecessor carries no weight. Start Stage 1 fresh.
- Opportunity: A leadership change at a previously locked account is a prime opportunity to establish a relationship before a competitor does. Move quickly.

Branch D: Contact Is Not a True Decision-Maker

- Indicators: Contact is friendly and engaged but referrals never materialize, they always defer to someone else

- Strategy: Ask — "Who else on your team is typically involved when a patient is being considered for hospice?" Then build a relationship with the identified stakeholder, keeping your current contact involved.
- Caution: Do not abandon a helpful contact even if they are not the decision-maker. Champions and influencers provide access you cannot get directly.

Branch E: Awareness or Knowledge Gap

- Indicators: Contact seems genuinely uncertain about when hospice is appropriate, asks basic eligibility questions, defers to physicians who are also uncertain
- Strategy: This is an education opportunity, not a sales problem. Offer a lunch-and-learn specifically on eligibility criteria for their primary patient diagnoses. Frame it as serving the clinical team, not as a sales visit.
- Outcome: Education-driven conversions tend to produce the most loyal long-term referral sources because the relationship started with genuine value delivery.

DECISION TREE 3: SHOULD I ASK FOR THE REFERRAL TODAY?

This is the question that stops many reps. They are afraid to ask too early and damage the relationship. They are equally afraid to wait too long and miss the moment. Use these criteria to make a clear decision.

Ask today if all of the following are true:

- ☐ You have completed meaningful Discovery and Connecting conversations with this contact
- ☐ They have acknowledged a specific patient need that your hospice can address
- ☐ There are no major unresolved objections or concerns that they have raised
- ☐ They have responded positively to your most recent educational content or visit
- ☐ You know of at least one patient in the facility who may currently be eligible

Do NOT ask today if any of the following are true:

- ☐ You are still in Stage 1 Discovery and have not built individual rapport yet
- ☐ They have expressed a concern or objection that you have not yet addressed
- ☐ You do not know their name for the patient they would most likely refer
- ☐ The relationship has not yet reached the point where they would share clinical concerns candidly
- ☐ There is a current service quality issue that has not been resolved to their satisfaction

ASKING FOR THE REFERRAL: SCRIPTED LANGUAGE

"Based on what you shared about [specific situation from previous conversation], it sounds like [patient type or clinical situation] might be the right fit for what our team does. Is there anyone you are currently managing who you have been wondering about in terms of appropriateness for comfort-focused care? I am happy to just do an informal consult with your clinical team to see if it would be a good fit."

Notice: the ask is framed as a clinical consult, not as a referral request. This lowers the perceived commitment threshold and positions you as a clinical resource rather than a sales rep.

THE DECISION TREE MINDSET

Decision trees are most valuable when you are stuck or uncertain. Before every visit, ask yourself which branch you are on. Before every conversation, know which question you are trying to answer. Clarity of purpose in each interaction compounds into consistent results across your entire territory. The rep who always knows why they are in a room is the one who earns the most trust.